

Encephalopathy of Unclear Etiology: A Diagnostic Dilemma.

PMID: 34255957 | Indexed in PubMed

Drug reaction with eosinophilia and systemic symptoms (DRESS) is a potentially fatal condition caused by drug exposure resulting in hypersensitivity reaction with involvement of different organ systems. We present a case of a 65-year-old man with a recent history of right total knee arthroplasty complicated by wound infection on a regimen of vancomycin who was transferred to our hospital for further management of fever, rigors, altered mental status, acute hypoxic respiratory failure, acute kidney injury, and development of an erythematous rash. DRESS syndrome was considered definite in this patient according to the European Registry of Severe Cutaneous Adverse Reaction Criteria, also known as RegiSCAR. To our knowledge, metabolic encephalopathy associated with multiorgan dysfunction resulting from vancomycin-induced DRESS syndrome has not been reported. A thorough analysis of recent medication history is essential for the prompt identification and management of this condition.

An Uncommon Presentation of DRESS Syndrome Secondary to Leflunomide Use: A Case Report.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome is a constellation of symptoms that manifest as a result of certain medications. Several antipsychotics, antibiotics, and sulfa-containing drugs are known to be implicated in the etiology of DRESS syndrome. The clinical presentation of this disorder consists of a diffuse rash, lymphadenopathy, and systemic organ damage. Our patient presented with symptoms consistent with DRESS syndrome after being started on leflunomide, which is not commonly associated with DRESS. The diagnostic workup comprised of monitoring inflammatory markers on laboratory work, an excisional lymph node biopsy (to rule out malignancy), and a skin biopsy (to assess the etiology of the rash). Our patient received systemic steroids, dose-adjusted based on expert opinion. Further research is required to explore the association between leflunomide and DRESS and address guidelines for the management of DRESS.

Drug Reaction With Eosinophilia and Systemic Symptom (DRESS) Following Rifampicin Treatment: A Case Report.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) is an idiosyncratic severe cutaneous adverse reaction (SCAR) characterized by a skin rash with systemic involvement (e.g., hematological, solid organ abnormalities). Various medications, most commonly anticonvulsants (carbamazepine, phenytoin), antibiotics (vancomycin, amoxicillin), and sulfa drugs (dapsone, sulfasalazine), have been implicated. We report a case of a 75-year-old man with pulmonary tuberculosis under anti-tubercular treatment (ATT Category 1 as per the national guidelines of Nepal) presenting with rash, fever, liver dysfunction, and eosinophilia, a combination of features suggestive of DRESS. According to the national tuberculosis (TB) survey of 2018-2019, over 117,000 people in Nepal were living with TB, including 69,000 newly diagnosed people. In third-world countries, such as Nepal, with a high TB prevalence, and the Southeast Asian region (with a huge percentage of the global burden of

TB

incidence), the risk of life-threatening adverse drug reactions during ATT is high. However, a good response is seen if it is recognized early and on stopping ATT and receiving a course of steroids and emollients.

DRESS syndrome with cerebral vasculitis.

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DRESS (drug rash with eosinophilia and systemic symptoms) syndrome is a severe reaction triggered by drugs that manifests as pyrexia and eosinophilia with involvement of the skin and internal organs. We herein describe the case of a patient who developed hyperuricemia after receiving treatment for tuberculosis, then took allopurinol and developed DRESS syndrome with a contextual coma and right hemisindrome. This report describes for the first time the presence of vasculitic cerebral involvement in a patient with DRESS syndrome. The cerebral vasculitis responded to treatment, showing clinical and instrumental remission. In cases such as this, allergic cerebral vasculitis should be considered in the differential diagnosis because it can be treated if recognized early, thus leading to remission without the development of any sequelae.

Drug reaction with eosinophilia and systemic symptoms: An emergency medicine focused review.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) is a severe drug reaction associated with potentially severe morbidity and mortality. This narrative review evaluates the pathogenesis, diagnosis, and management of DRESS for emergency clinicians. DRESS is a severe hypersensitivity reaction which can involve multiple organ systems, including the skin, lymphatic system, lungs, kidneys, and heart. Treatment is primarily supportive care combined with removal of the offending agent and initiation of corticosteroids. Additional options include cyclophosphamide, cyclosporine, intravenous immunoglobulin, and N-acetylcysteine. It is important for the emergency clinician to be aware of the diagnosis and management of DRESS in order to best optimize care for these patients.

Drug Reaction with Eosinophilia and Systemic Symptoms (DRESS): A National Analysis of Data from 10-Year Post-marketing Surveillance.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) is a rare, severe and potentially fatal cutaneous adverse drug reaction (the mortality rate is up to 10 %) associated with numerous and apparently heterogeneous drugs. The aetiology is unknown. To report Italian cases of DRESS over a 10-year period. We searched the National Pharmacovigilance Network (NPN) for the term 'drug reaction

with eosinophilia and systemic symptoms' from 1 January 2004 to 1 January 2014, to identify all reports of DRESS. Each case was checked to avoid duplication. In the NPN, we identified 91 serious cases of DRESS: 68 were spontaneous, still-unpublished reports, while 23 additional cases were derived from screening of the scientific literature, performed by marketing authorization holders. Notably, the single common element linking all cases of DRESS was intake of a drug containing an aromatic ring. Thanks to the largest national DRESS case series ever reported, we were able to hypothesize, for the first time, that there is an association between use of drugs containing an aromatic ring in their chemical structure and DRESS. This might aid understanding of the aetiology of DRESS and facilitate diagnosis.

[DRESS syndrome].

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DRESS syndrome (Drug Rash with Eosinophilia and Systemic Symptoms) is severe drug-induced allergic-type reaction which occurs few days to weeks after taking a drug in a predisposed patient. Organ damage, eosinophilia and skin rash are typical at presentation. Corticotherapy is often necessary in severe cases. In this report we describe a case of 56-year old female with fever, elevated liver tests and skin rash. DRESS syndrome was diagnosed and allopurinol was indentified as a causative drug. Due to possible fatal outcome, DRESS syndrome should be considered in a differential diagnosis of all patients presenting with similar signs and symptoms.

Drug reaction with eosinophilia and systemic symptoms syndrome associated with Nitrofurantoin.

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Drug reaction with eosinophilia and systemic symptom (DRESS) is a severe adverse drug-induced reaction with a prolonged latency period which is characterized by a variety of clinical manifestations, usually fever, rash, lymphadenopathy, eosinophilia, and a wide range of mild-to-severe systemic presentations. Drugs are an important cause of DRESS in most of the cases. It is challenging to diagnose DRESS because of the diversity of cutaneous eruption and visceral organs involvement. We hereby report a 34-year-old female who developed DRESS syndrome following ingestion of nitrofurantoin for the treatment of urinary tract infection. She was managed conservatively and recovered after few weeks. Our aim of this study is to raise awareness to suspect DRESS syndrome in patients who present with unusual clinical features with skin involvement after initiating any drug.

[Management of drug reaction with eosinophilia and systemic symptoms (DRESS)].

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The management of drug-induced hypersensitivity syndrome or drug reaction with eosinophilia and systemic symptoms (DRESS) is not codified. Demonstration of the reactivation of Herpesviruses illustrates the specific pathophysiology of this syndrome. Proposals for the management of DRESS were elaborated by the cutaneous adverse drug reaction working group of the French Society of Dermatology

to help with its management. From a review of literature and the experience of the members of this group, consensual proposals were written about diagnostic criteria, tests, treatment options, and follow-up. These proposals will need to be validated in prospective studies. A decisional tree of treatment options is proposed, based on the severity of visceral manifestations. The importance of a rapid withdrawal of the culprit drug and of a long-term follow-up is underlined. Treatment will be adapted to the clinicobiological status (topical corticosteroid, systemic corticosteroid, intravenous gammaglobulins, antivirals).

Minocycline-induced drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome with persistent myocarditis.

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Drug reaction with eosinophilia and systemic symptoms (DRESS) syndrome is a rare serious adverse effect associated with a variety of medications. We present a case of minocycline-induced DRESS syndrome, which resulted in acute renal failure, transient thyroiditis, and transaminitis, and a persistent lymphocytic myocarditis resulting in congestive heart failure. To our knowledge, this is the third reported case of minocycline-induced myocarditis. Additionally, we report successful plasmapheresis and rituximab treatment for minocycline-induced myocarditis associated with the DRESS syndrome.
